

Measuring Stress and Its Effects on Health

Most investigators agree that severe stress can endanger a person's health. For example, prolonged job stress is significantly correlated with anxiety and depression (deWolff, 1985). People who experience severe stress on the job report frequent illnesses as well. (At least, they often call in sick and stay home from work!)

How much stress is injurious to one's health? Is it true that the more stress a person experiences, the more that person's health suffers?

To answer such questions, we need to measure both stress and health. Measuring health is tough enough; measuring stress is even more difficult. One approach is to give people a checklist of stressful experiences. For example, Thomas Holmes and Richard Rahe (1967) devised a Social Readjustment Rating Scale (Table 12.2) that assigns points for both desirable and undesirable events, in accordance with Selye's idea that any change in a person's life is stressful. Note, for example, that you could get 35 points for "change in number of arguments with spouse"—the same number of stress points for an increase or a decrease in arguments! To measure the amount of your stress, you are

supposed to check off all the experiences you have had within a given period of time, such as the last 6 months, and total up the points assigned to each.

Although this scale has been widely used, it is subject to many criticisms: First, the assumption that we can simply add stress points from various events is probably incorrect (Birnbaum & Sotoodeh, 1991). For example, suppose you graduate from college (26 points), treat yourself to a vacation (13), and then start a new job (36). That adds up to 75 points—more than you would get for a divorce (73) or the death of a close family member (63).

Second problem: The scale includes 53 points for suffering the stress associated with "personal injury or illness." It also ascribes points for sexual difficulties, change in sleeping habits, and change in eating habits—all symptoms of illness. So, it is hardly surprising that people with high scores on this rating scale have an increased probability of being ill.

Third, the scale fails to measure some important stressors. Selye defined stress as a response to changes in one's life, and accordingly, this scale ignores the stress of coping with ongoing problems, such as racism or poverty. It also ignores the stress we experience from events that almost happened. For example, all year long you expect to

TABLE 12.2 Social Readjustment Rating Scale

RANK	LIFE EVENT	POINT VALUE	RANK	LIFE EVENT	POINT VALUE
1	Death of spouse	100	22	Change in responsibilities at work	29
2	Divorce	73	23	Son or daughter leaving home	29
3	Marital separation	65	24	Trouble with in-laws	29
4	Jail term	63	25	Outstanding personal achievement	28
5	Death of close family member	63	26	Wife begin or stop work	26
6	Personal injury or illness	53	27	Begin or end school	26
7	Marriage	50	28	Change in living conditions	25
8	Fired at work	47	29	Revision of personal habits	24
9	Marital reconciliation	45	30	Trouble with boss	23
10	Retirement	45	31	Change in work hours or conditions	20
11	Change in health of family member	44	32	Change in residence	20
12	Pregnancy	40	33	Change in schools	20
13	Sex difficulties	39	34	Change in recreation	19
14	Gain of new family member	39	35	Change in church activities	19
15	Business readjustment	39	36	Change in social activities	18
16	Change in financial state	38	37	Mortgage or loan less than \$10,000	17
17	Death of a close friend	37	38	Change in sleeping habits	16
18	Change to different line of work	36	39	Change in number of family get-togethers	15
19	Change in number of arguments with spouse	35	40	Change in eating habits	15
20	Mortgage over \$10,000	31	41	Vacation	13
21	Foreclosure of mortgage or loan	30	42	Christmas	12
			43	Minor violations of the law	11

SOURCE: Holmes & Rahe, 1967.

be laid off from your job. You keep waiting, but the rumored plant closing never happens. Or you have been counting on a promotion at work, and you have told all your friends that you are expecting it, but then you do not get promoted. You will get no points on the rating scale for “not getting fired” or “not being promoted.” And yet anyone who has lived through such an experience can tell you that it was very stressful.

SOMETHING TO THINK ABOUT

Can you think of a better way to measure stress? Some psychologists have given people a list of events and have asked them to check off events they have experienced and also to assign a value to each event, on the basis of how stressful they found it to be. Is that an improvement, or does it introduce problems of its own? ★

Lazarus's Approach to Stress

Aside from the problems with the Social Readjustment Rating Scale, however, many psychologists challenge the basic assumption that a particular life change necessarily produces a specific amount of stress and that a given event will be equally stressful for all people. As Richard Lazarus (1977) pointed out, the amount of stress evoked by an event depends on how people interpret the event and what they can do about it. Pregnancy can be much more stressful for a 16-year-old unmarried woman than it is for a 27-year-old married woman. Being fired may be a disaster for a 50-year-old who expects to have trouble finding another job; it may be only a minor annoyance to a 19-year-old.

According to Lazarus, a stressful situation is one that someone regards as threatening and as possibly exceeding his or her resources (Lazarus, 1977). (See Figure 12.25.) Note that this view of stress assumes a major role for people's knowledge, experience, and cognitions: If you are bitten by a snake, you might remain calm if you recognize the snake as a harmless variety. If your boss criticizes your work, you might shrug off the insult if you know your boss had reason to be in a bad mood.

To the extent that stress depends on our interpretation of an event, not simply on the event itself, people can learn to cope with potentially stressful events, as we shall see later in this chapter. People can learn to deal with events actively instead of feeling threatened by them. Given this view, the proper measure of stress would include not only the unpleasant events (“hassles”) that we have to deal with but also the pleasant events (“uplifts”) that brighten our day and help to cancel out the unpleasant events (Kanner, Coyne, Schaefer, & Lazarus, 1981). Table 12.3 presents one example of this approach.

In short, stress is difficult to measure accurately. Nevertheless, we can identify particular kinds of stressful experiences that endanger many people's health. Study-



FIGURE 12.25 Lazarus stated that evaluation of some kind, conscious or unconscious, always precedes emotion. Thus, a given event can be highly stressful for one person yet only slightly stressful or not at all for a second person.

ing the effects of experience on health and illness can offer us insights into the relationships among stressful experiences, interpretation of those experiences, and body functioning.

TABLE 12.3 Ten Most Frequent Hassles and Uplifts

HASSLES	UPLIFTS
1. Concerns about weight	1. Relating well with your spouse or lover
2. Health of a family member	2. Relating well with friends
3. Rising prices of common goods	3. Completing a task
4. Home maintenance	4. Feeling healthy
5. Too many things to do	5. Getting enough sleep
6. Misplacing or losing things	6. Eating out
7. Yard work or outside home maintenance	7. Meeting your responsibilities
8. Property, investment, or taxes	8. Visiting, phoning, or writing someone
9. Crime	9. Spending time with family
10. Physical appearance	10. Home (inside) pleasing to you

SOURCE: Kanner, Coyne, Schaefer, & Lazarus, 1981.

Stress and Psychosomatic Illness

A **psychosomatic illness** is not an imagined or a feigned illness. Rather, it is an illness that is influenced by a person's experiences—particularly stressful experiences—or by his or her reactions to those experiences. That is, something about the person's behavior or way of life influenced the onset and progression of the disease. Most illnesses are partly psychosomatic in this sense.

For many years, physicians looking for the sources of illness concentrated on physical agents such as germs or injuries, giving no thought to the possibility of a psychosomatic influence. Then, in the early 1800s, they found soldiers who were suffering from what we would now call posttraumatic stress disorder. Some of them showed serious (though temporary) physical ailments, including blindness or paralysis, even though they had never been injured. A few even died on the battlefield when a cannonball landed nearby without striking them. Physicians of the time suggested that the soldiers were injured by the wind of the cannonball passing by, or by atmospheric electricity stirred up by the wind, or by the heat or the temporary vacuum it left in its wake. Today, those hypotheses sound extremely far-fetched; at the time, they seemed more reasonable than the “ridiculous” idea that mere fear or other psychological states could influence someone's health (McMahon, 1975).

Physicians and psychologists still have difficulty explaining how emotional states affect the body. They do not assume that emotions lead directly to illness. They know, however, that people who have certain emotional experiences are more likely than others to overeat, to smoke, or to engage in other habits that increase the risk of illness. Certain behaviors and experiences can damage the immune system and increase a person's vulnerability to a variety of disorders, ranging from minor infections to cancer (Shavit et al., 1985).

One young woman died of fear in a most peculiar way: When she was born, on Friday the 13th, the midwife who delivered her and two other babies that day announced that all three were hexed and would die before their 23rd birthday. The other two did die young. As the third woman approached her 23rd birthday, she checked into a hospital and informed the staff of her fears. The staff noted that she dealt with her anxiety by extreme hyperventilation (deep breathing). Shortly before her birthday, she hyperventilated to death.

How did this happen? Ordinarily, when people do not breathe voluntarily they breathe reflexively; the reflex is triggered by carbon dioxide in the blood. By extreme hyperventilation, this woman had exhaled so much carbon dioxide that she did not have enough left to trigger reflexive breathing. When she stopped breathing voluntarily, she stopped breathing altogether (“Clinicopathologic conference,” 1967). This

is a clear example of a self-fulfilling prophecy: The fact that the woman believed in the hex caused it to be fulfilled. This is also a clear example of the indirect influence of emotions on health.

We shall examine heart disease and cancer as examples of diseases that may be linked to particular emotional experiences or personality types. In both cases, the evidence has both strengths and weaknesses.

Heart Disease

An upholsterer repairing the chairs in a physician's waiting room once noticed that the fronts of the seats wore out before the backs. To figure out why, the physician began watching patients in the waiting room. He noticed that his heart patients habitually sat on the front edges of their seats, waiting impatiently to be called in for their appointments. This observation led the physician to hypothesize a link between heart disease and an impatient, success-driven personality, now known as the Type A personality (Friedman & Rosenman, 1974).

People with **Type A personality** are highly competitive; they believe that they must always win. They are impatient, always in a hurry, and often angry and hostile. By contrast, people with a **Type B personality** are relatively easygoing, less hurried, and less hostile. For example, Gary Schwartz (1987) described his observations of two men fishing: One (a Type B) slowly baited his hook, dropped his line into the water, and sat back watching the gulls and waiting for a bite. Another man (a Type A), fishing with two poles, spent much of his time rushing back and forth between the two poles, cursing when the two lines got tangled with each other. When another fisher caught a large fish, this man pulled up his anchor in frustration and raced his boat off to another part of the bay. (Are you a Type A or a Type B? Test yourself by answering the questions in Figure 12.26.)

TRY IT
YOUR-
SELF

Statistically, a link does exist between Type A personality and susceptibility to heart disease. People with a Type A personality tend to have a higher heart rate and higher blood pressure than most other people. The difference is larger in competitive situations, where people with a Type A personality show great increases in heart rate and blood pressure (Lyness, 1993).

Additional evidence for this linkage comes from studies comparing heart disease in various cultures (Levine, 1990). Some cultures have a hurried pace of life; people walk fast; they talk fast; almost everyone wears a watch; storekeepers pay prompt attention to their customers. Other cultures have a more relaxed pace of life; people are seldom in a rush; few people wear watches; the buses and trains seldom arrive on schedule but no one seems to care (Figure 12.27). As you might guess, the rate of heart disease is higher in countries with a hurried pace of life than it is in countries with a more relaxed pace. In the United States, the risk of

Measuring the Type A Personality

- _____ 1. Do you find it difficult to restrain yourself from hurrying others' speech (finishing their sentences for them)?
- _____ 2. Do you often try to do more than one thing at a time (such as eat and read simultaneously)?
- _____ 3. Do you often feel guilty if you use extra time to relax?
- _____ 4. Do you tend to get involved in a great number of projects at once?
- _____ 5. Do you find yourself racing through yellow lights when you drive?
- _____ 6. Do you need to win in order to derive enjoyment from games and sports?
- _____ 7. Do you generally move, walk, and eat rapidly?
- _____ 8. Do you agree to take on too many responsibilities?
- _____ 9. Do you detest waiting in lines?
- _____ 10. Do you have an intense desire to better your position in life and impress others?

FIGURE 12.26 If you answer yes to a majority of these items, Friedman and Rosenman (1974) would say that you probably have a Type A personality. But they would also consider your explanation of your answers, so this questionnaire gives only a rough estimate of your personality. Friedman and Rosenman classified everyone as either Type A or Type B, but most psychologists believe that people can exhibit various degrees of Type A traits.

heart attacks and the pace of life are generally highest in large northeastern cities; they are lowest in the small towns in the west and the south. Of course, these are correlational data; they do not demonstrate conclusively that a frantic pace of life causes heart problems. (For example, something about cultural differences in diet or climate might simultaneously influence people's activity levels and their heart muscles.)

At first, researchers focused on the role of impatience and competitiveness as the main links between Type A behavior and heart disease. However, later evidence has indicated that heart disease correlates more strongly with unpleasant emotions, especially hostility. A hostile attitude is only a small contributor to heart disease, however, in comparison to genetics, diet, exercise, and other factors (Miller, Smith, Turner, Guijarro, & Hallet, 1996). Still, even a small contributor to heart disease is worth taking seriously.

CONCEPT CHECK

4. People with a Type A personality are likely to develop stress-related heart disease. Yet, when they fill out the Social Readjustment Rating Scale, mentioned earlier, their scores are often low. Why might that scale understate the stress levels of Type A people? (Check your answer on page 466.)

Cancer

There are many kinds of cancer and many causes that are not yet well understood. Behavior can also influence the onset and spread of cancer, at least indirectly. For example, women who examine their breasts regularly can detect breast cancer at an early stage, when treatment is more likely to be successful. However, most women do not conduct regular, competent self-examinations, either because they do not know how, or because they are fearful, or because they feel helpless to do any good (Miller, Shoda & Hurley, 1996). Latina women in the United States have a higher rate of cervical cancer than other women, partly because they are less likely than other women to see doctors regularly for Pap smear tests (Meyerowitz, Richardson, Hudson, & Leedham, 1998). In many ways, preventing or treating cancer requires behavioral as well as medical interventions.



FIGURE 12.27 People in some cultures have a frantic pace of life: Everyone seems to be in a rush; people walk fast, talk fast, and push one another around. In other cultures, no one is sure what time it is and no one cares. The risk of heart disease is greatest in cultures or subcultures with a frantic pace of life.

Do our emotions contribute directly to cancer? Because the brain influences the immune system, which fights cancer, an emotional experience might lead to an impairment of the immune system and therefore to a greater risk of certain kinds of cancer.

The two emotional states most likely to lead to cancer are depression and stress. Many cancer patients are depressed (Weinstock, 1984), and many of them report that they were depressed, often following the death of a loved one, long before they knew they had cancer. Severe depression suppresses the activity of the immune system and leaves a person more vulnerable than usual to all sorts of infections and diseases, including the spread of certain types of tumors (Anisman & Zacharko, 1983; Baker, 1987).

While conducting research on the effects of stress in animals, investigators have found that stress increases the spread of cancer and shortens the animal's lifespan. But it is difficult to extrapolate from those results to humans. First, the results vary from one study to another, depending on the duration and type of stress and the genetic makeup of the animals. Second, nearly all animal studies deal with cancers caused by viruses, and viruses cause fewer than 5% of human cancers (Fox, 1983).

Depression, stress, and severe emotional problems probably do increase the risk of cancer in humans. Still, their influence is minor; emotional factors are far less important in causing cancer than are genes and toxic substances (Anisman & Zacharko, 1983; Derogatis, 1986; Fox, 1983). Keeping a positive outlook on life may help to prevent cancer; still, many people suffering from serious, long-lasting depression manage to survive into old age (Stein, Miller, & Trestman, 1991).

Psychological factors exert a stronger influence on what happens after the onset of cancer. For many people, the stress of dealing with cancer decreases the pleasures of life, impairs eating and sleeping, and directly or indirectly weakens the ability of the immune system to attack the cancer cells (Andersen, Kiecolt-Glaser, & Glaser, 1994). People who receive steady support from their family and friends, or from a psychotherapist or a self-help group, have a better chance of recovery and a better quality of life while they are fighting the disease (Fawzy, Fawzy, Arndt, & Pasnau, 1995).

THE MESSAGE

Health is Both Psycho and Somatic

The important point to remember about health psychology is that personality, emotions, and behavior are just one aspect of health. Good diet, good exercise, and quitting tobacco improve one's chances for good health; regular breast self-exams help to detect possible problems early; controlling hostility and other negative emotions will decrease the

risk of heart disease. Still, we should not set unrealistic expectations for behavioral strategies. No matter how much someone follows the best psychological advice, health is also subject to the influence of genetics, toxic substances in the environment, and other factors that are hard to control. So, yes, do what you can to improve your behavior and attitude, but if problems arise, don't expect attitude readjustment to take the place of medical help. And if you become ill, don't feel guilty that you just didn't have the right attitude. What happens to your health is partly under your control, but not entirely.

S U M M A R Y

★ *Selye's concept of stress.* According to Hans Selye, stress is "the nonspecific response of the body to any demand made upon it." Any event, pleasant or unpleasant, that brings about change in a person's life produces some measure of stress. This theory, though influential, has some problems and limitations. (page 459)

★ *Stages of response to stress.* The body goes through three stages in response to a stressful experience: alarm, resistance, and exhaustion. In the resistance and exhaustion stages, prolonged channeling of energy toward resisting stress can weaken the immune system. (page 459)

★ *Influence of past experiences.* The degree of stress that an event evokes depends not only on the event itself but also on the person's interpretation of that event. People with posttraumatic stress disorder react strongly to daily events because of their previous experiences with war, rape, or other deeply upsetting occurrences. (page 460)

★ *Difficulties of measuring stress.* The stress that an individual experiences is difficult to measure. Two people who have gone through similar experiences may show different levels of stress. (page 460)

★ *Psychosomatic illness.* Stress, hostility, and other emotional experiences may increase the probability of certain illnesses. A psychosomatic illness is somehow related to a person's experiences or to his or her reactions to those experiences. (page 463)

★ *Heart disease.* People with a Type A personality are competitive, impatient, and hostile. They are more likely than others to suffer heart disease, although the strength of that relationship and the reasons behind it are still in dispute. The emotional states of depression and hostility pose a greater risk than competitiveness and impatience. (page 463)

★ *Cancer.* Depression and stress can increase the risk of cancer, at least slightly. Social support improves people's chances of recovery from cancer and also improves their quality of life while fighting the disease. (page 464)

Suggestion for Further Reading

Brannon, L., & Feist, J. (1997). *Health psychology* (3rd ed.).

Belmont, CA: Wadsworth. A textbook that surveys the relationship between behavior and health.

Terms

- health psychology** a field of psychology concerned with how people's behavior can enhance health and prevent illness and how behavior contributes to recovery from illness (page 459)
- stress** according to Hans Selye, the nonspecific response of the body to any demands made upon it; according to Lazarus, a situation that someone regards as threatening and as possibly exceeding his or her resources (pages 459)
- alarm** the first stage of response to stress, a brief period of high arousal of the sympathetic nervous system, readying the body for vigorous activity (page 459)
- resistance** the second stage of response to stress, a stage of prolonged but moderate arousal (page 460)
- exhaustion** the third stage of response to stress, when the body's prolonged response to stress decreases the synthesis of proteins, including the proteins necessary for activity of the immune system (page 460)
- posttraumatic stress disorder (PTSD)** a condition in which people who have endured extreme stress feel prolonged anxiety and depression (page 460)
- psychosomatic illness** an illness that is influenced by a person's experiences—particularly stressful experiences—or by his or her reactions to those experiences (page 463)

Type A personality a personality characterized by constant competitiveness, impatience, anger, and hostility (page 463)

Type B personality a personality characterized by an easy-going attitude, with little hurry or hostility (page 463)

Answer to Concept Check

- The Social Readjustment Rating Scale measures events that change a person's life; it does not measure constant sources of stress such as the pressures of work. The scale also fails to measure people's reactions, such as impatience, competitiveness, and hostility. (page 464)

Web Resources

Lifescan Health Risk Appraisal

wellness.uwsp.edu/Health_Service/services/lifeScan.shtml

Do you know what your *appraised health age* is? Answer a few health questions and find out. If possible, first check your blood pressure, total cholesterol, and HDL cholesterol levels.

Coping with Stress

How can we reduce the harmful effects of stress on the body?

How can we learn to cope with stress?

An eccentric millionaire whom you have never met before hands you a \$10 bill for no apparent reason, no strings attached. How do you feel? Pleased, I assume.

Now let's change the circumstances a bit: The millionaire had been handing out \$100 bills until it was your turn, but then: "Sorry, I just ran out of \$100 bills. So I'll have to give you a \$10 bill instead." Now how do you feel? disappointed, sad, angry? You may even feel *cheated*, even though you have just received something for nothing.

Just as your reaction to a gift of \$10 depends on the circumstances, so does your reaction to bad news. How would you feel if you had studied hard for a test and then got a C-? Unhappy, I presume. But if you then discovered that everyone around you had failed the test, you would begin to feel much better about your C-.

How you feel about an event depends not just on the event itself but also on how you interpret it (Frijda, 1988; Lazarus, Averill, & Opton, 1970). Was it better or worse than you had expected? Better or worse than what happened to other people? Was it a one-time event, or did it carry a hint of what might happen in the future? How you feel about an event also depends on your personality. Some people manage to keep their spirits high even in the face of tragedy; others are devastated by minor setbacks.

Coping with stress is the process of developing ways to decrease its effects and to get through difficult tasks despite the stress. How do people cope with disappointments, anxieties, and stress? And can we learn to cope more successfully?

Coping Styles and Strategies

People cope with stress in a great many ways, but we can group most of them into two major categories. In the style known as monitoring, one attends carefully to the stressful event and one's reaction to it and tries to take effective action. In blunting, one tries to avoid the stressful event or at least avoid thinking about it. Some people tend to rely more on one style than on the other, but it is best to alternate between the two depending on circumstances, as most people do (Heszen-Niejodek, 1997). In particular, when the stressful event is controllable, the best policy is to take action, using the monitoring approach. For example, if you have test-taking anxieties, you could try to relax and not think about the test, but if you instead spend time studying for the test, you will probably improve your grade and reduce your anxiety. Similarly, if you have troubles in your marriage or with your job, the best way to reduce the stress is to search for a solution to the problem.



The stressfulness of an event depends on how we interpret it, not just on the event itself. For example, most people would be delighted to finish second or third in an Olympic event, but someone who was expecting to finish first may consider any lesser result to be a defeat and a disappointment.



"Cabin fever"—the distress people feel when they are cooped up and cannot leave—is largely due to one's feeling unable to predict or control events.

However, suppose your stress comes from having to lie in a hospital bed awaiting an operation or recovering from one. Or having to sit in a dentist's chair for some unpleasant procedure. Now, you have few active responses that will make much difference. You can't run away or attack, for example; you just have to sit there and endure the situation. In such cases, one must turn to a blunting strategy.

Monitoring Strategies

Whenever a stressful situation is at all controllable, people feel better if they take action. In fact, they feel better even if the action they take is ineffective, as long as they think it might be effective. Here, we will consider the importance of a sense of control and then discuss a few additional monitoring strategies.

The Importance of Predictability and Control

Let's start by contrasting two hypothetical situations. First, imagine that some torturer is keeping you awake 24 hours per day. You have no way of escaping and no way of knowing how long the torture will last, but in fact it lasts 3 days until someone finally liberates you. Contrast that experience with an experiment in which you agree to go without sleep for 3 days. You could quit if you wanted to, but in fact you don't.

Another comparison: An enormous snowstorm has trapped you in a small cabin. You have an ample supply of food and fuel, but you have no idea how long you will be stuck in the cabin. In fact, the snow melts 5 days later, thus allowing you to leave. Contrast that with a case where you decide to isolate yourself in a cabin for 5 days so you can finish a painting or a musical composition.

In both cases, the physical circumstances are the same—3 days without sleep or 5 days without leaving a cabin. But if you are doing something voluntarily, if you know what to expect, and if you know that you could quit if you wanted to, the events are far less stressful. When hospital patients or nursing-home residents are told what to expect and are given the opportunity to make even a few choices, they feel better and on the average live longer (Shapiro, Schwartz, & Astin, 1996). Many people find that religious faith helps them to cope with stress by giving them some sense of control over matters that are otherwise uncontrollable.

In several experiments, people were asked to perform difficult tasks while listening to loud bursts of noise that might impair their performance. Some participants had no power to control the noise; others were shown a switch that could cut off the noise, although the experimenters preferred that the subjects not use that switch unless it became necessary. Even though none of the subjects flipped the switch, simply knowing that they *could* turn off the noise made it less disturbing to them. They performed better on a proofreading task than did the subjects with no sense of control (Glass, Singer, & Pennebaker, 1977; Sherrod, Hage, Halpern, & Moore, 1977).

Why do you suppose people who consistently blame other people for their misfortunes tend to have a poor psychological adjustment (Tennen & Affleck, 1990)? One explanation is that, if you think other people are maliciously thwarting your plans, you no longer feel in control of the situation.

CONCEPT CHECK

5. Which would disrupt your studying more, your own radio or your roommate's radio? Why? (Check your answer on page 473.)



Practice at self-defense serves as a kind of inoculation against fear. The thought of being attacked is less frightening to one who has some idea of how to handle the situation.

Why does a predictable or controllable event produce less stress than an unpredictable or uncontrollable event does? One explanation is that we fear an unpredictable, uncontrollable event may grow so intense that it will eventually become unbearable (Thompson, 1981). As long as we know what will probably happen next, we assume that things won't get any worse. And as long as we have some measure of control over an event, we tell ourselves that we can take action if the situation becomes unbearable.

A second explanation is that, when an event is predictable, we can prepare for it at the appropriate time and relax at other times. If you dread being called upon to answer questions in class and your professor calls on students at random, you have to remain tense and ready at all times. But if your professor calls upon students in alphabetical order, you can relax until shortly before your turn comes, and then get ready.

Inoculation

A stressful experience is less disturbing if you know what to expect, but it is hard to know what to expect if you have not been through the experience before. Sometimes, a good solution is to provide people with a small-scale preview of a stressful experience that they may need to face later. In other words, we can “inoculate” or immunize someone against certain kinds of stressful experiences.

One way to **inoculate** people against stressful events is to expose them to smaller amounts of such events beforehand (Janis, 1983; Meichenbaum, 1985; Meichenbaum & Cameron, 1983). For example, many armies have

soldiers practice combat skills under realistic conditions, sometimes under actual gunfire. Another way is through role-playing. A police trainee might pretend to intervene while two people act like a husband and wife engaged in a violent quarrel. If you are nervous about going to your landlord with a complaint, you might get a friend to play the part of the landlord and then practice what you plan to say.

Inoculation has proved successful with young people suffering from “dating anxiety.” Some young people are so nervous about saying or doing the wrong thing that they avoid all opportunities to go out on a date. By means of role-playing, they can practice dating behaviors with assigned partners and thus feel less apprehensive about dating (Jaremko, 1983).

CONCEPT CHECK

- Suppose you are nervous about giving a speech before a group of 200 strangers. How could you inoculate yourself to reduce the stress? (Check your answer on page 473.)

Social Support

Have you ever had a secret too shameful to tell? Have you stopped yourself from disclosing a personal experience because you thought others would think less of you? ... Have you ever lied to yourself by claiming that a major upheaval in your life didn't affect you or, perhaps, didn't occur? If so you may be hurting yourself. Not because you have had a troubling experience but because you can't express it.

—JAMES W. PENNEBAKER (1990)



Simply writing about one's deepest feelings or painful experiences helps relieve the tension. It demonstrates that the events were not too painful even to think about.

A great many people have undergone a severely painful experience at one time or another. Perhaps you were beaten or sexually molested; perhaps you were responsible for someone else's injury; perhaps you attempted suicide; perhaps someone humiliated you in public. Whatever your experience, you may decide that it was too painful even to think about, much less talk about. Therefore, the pain builds up inside.

People who have had painful experiences report that they feel much better after they have a chance to talk about them to someone, or even just write about them. In one series of experiments, college students were randomly assigned to two groups. For about half an hour per day on 3–5 consecutive days, the experimental group was asked to write about their deepest thoughts and feelings about an emotional issue that had affected them. For example, many wrote about the death of a loved one, a personal failure, or sexual or physical abuse. The psychologists conducting the study said nothing to them about what they had written, and in fact kept no record of which student had written which essay. The control group spent the same amount of time writing about unemotional topics, such as how they budgeted their time in a typical day. The experimental group described the experience as extremely valuable, in spite of the fact that many of them had become very upset, even tearful, during the experiment. A follow-up measurement found that the experimental group members had better health over the next few months, got better grades, and drank less alcohol (Pennebaker, 1997). Evidently, just expressing their feelings reduced their stress and enabled them to relax and deal more effectively with life.

In many cases, people gain the greatest support by talking with people who have lived through similar crises. Alcoholics Anonymous, for example, is composed of recovering alcoholics who try to help one another. To give another example, the nurses who work in the intensive care units of hospitals undergo constant, often severe stress (Hay & Oken, 1977). The unspoken (but rigidly obeyed) rule is never to refuse help to either a patient or a fellow nurse. These nurses are surrounded by the sights, sounds, and smells of patients who are suffering and dying. When a patient dies, it is often the nurse, not the doctor, who must inform the relatives. At the end of the day, these nurses cannot simply go home and resume life as usual. They need to unwind, and they often do so by sharing their experiences with other nurses. Receiving social support, especially from sympathetic people who have endured similar problems, improves overall health and actually increases life expectancy (Uchino, Cacioppo, & Kiecolt-Glaser, 1996).

Beliefs as Coping Strategies

- In the long run, I shall be more successful than most.
- Sure, I have my strengths and weaknesses, but my

strengths are in areas that are important; my weaknesses are in areas that don't really matter.

- When I fail, it is because I didn't try hard enough or because I got some bad breaks—not because of any lack of ability.
- No matter how bad (or good) things are, they are going to get better.
- Right now I'm sad that my wife (husband) left me, but in the long run I'll be better off without her (him).
- I lost my job, but in many ways it was a crummy job. The more I think about it, the happier I am about it. I can get a better one.

Any of these statements may be correct or incorrect for a given person at a given time. Remember the “personal fable” of adolescence from Chapter 10? Most normal, happy people nurture various versions of that fable throughout life. They emphasize their strengths, downplay their weaknesses, and distort bad news to make it seem not so bad, maybe even good (Taylor & Brown, 1988). To some extent, these beliefs can help people to deal with the difficult and stressful times of life.

When a situation is undeniably bad, people can still find ways to deny the problem. Medics serving in the Vietnam War knew they were risking their lives every time they boarded a helicopter to go to the aid of wounded soldiers. Because they had no way to predict or control the enemy's actions, we might expect them to have experienced extreme stress. In general, however, measurements of the activity of their autonomic nervous system showed low levels of arousal on both flight days and nonflight days (Bourne, 1971). Why? They had managed to convince themselves of their own invulnerability. They even told exaggerated stories about their close brushes with death as evidence that they led a charmed life.

People also look for an optimistic outlook to help them cope with severe illness. One study of 78 women who had surgery for the removal of breast cancer found that they coped with their stress and anxiety in three ways (Taylor, 1983):

First, they searched for meaning in the experience. Many of them said that they had become better people and had developed a new attitude toward life.

Second, they tried to regain a feeling of mastery over their lives. They wanted to believe that they knew why they had developed cancer and that they knew how to avoid a recurrence.

Third, they all sought to boost their self-esteem by comparing themselves with someone else who was worse off: “I'm glad I had only one breast removed instead of both, like some other women.” “Sure, I had both breasts removed, but at least I was 70 years old at the time. It would have been worse if it had happened when I was young.” “It was terrible to have both breasts removed at age 35, but I was already married, and my husband has been sympathetic and supportive.”



Relaxation and exercise are strategies that people can use to manage their reactions to problems. People who devote a short time each day to relaxation report less stress, and exercise can be a way of working off excess energy, allowing greater relaxation.

In short, people tell themselves: “I’m in control; my life is getting better; at least, things are better than they might have been.” Is it good for our mental health for us to remain unrealistically optimistic? On that question, psychologists disagree (Colvin & Block, 1994; Taylor & Brown, 1994). Certainly, some people manage to maintain excellent mental health without maintaining any apparent illusions about themselves. Although researchers have not yet measured the limits of optimistic thinking, it seems likely that the best general attitude might be *slightly* unrealistic optimism.

Blunting Strategies

Blunting strategies do not attempt to solve the underlying problems, but they help us to manage our reactions to them. Three common examples are relaxation, exercise, and distraction.

Relaxation

Sometimes people have a real problem, such as an impending medical operation, but they cannot do much about it. At other times, people get nervous even when they have no real problem (Manuck, Cohen, Rabin, Muldoon, & Bachen, 1991). In such situations, it is helpful to try to relax. Here are some suggestions that may help (Benson, 1985):

- Find a quiet place. Do not insist on absolute silence; just find a spot where the noise is least disturbing.
- Adopt a comfortable position, relaxing your muscles. If you are not sure how to do so, start with the opposite: *Tense* all your muscles so you become fully aware of how they feel. Then relax them one by one, starting from your toes and working systematically toward your head.
- Reduce sources of stimulation, including your own thoughts. Focus your eyes on a simple, unexciting object. Or repeat something over and over—a sentence, a phrase, a prayer, or even a meaningless sound like *om*—whatever feels comfortable to you.
- Don’t worry about anything, not even about relaxing. If worrisome thoughts keep popping into your head, dismiss them with an “oh, well.”

Some people call this practice meditation. People who practice this technique daily report that they feel less stress. Many of them improve their overall health (Benson, 1977, 1985). One study found that people who went through a 12-week meditation program had a long-lasting decrease in anxiety and depression, as compared with a control group who spent the same amount of time listening to lectures about how to reduce stress (Sheppard, Staggers, & John, 1997).

Another step toward relaxation is to learn to interpret situations realistically. Some people fret forever about disasters that *might* happen or about something someone said that *might* be taken as an insult. Psychologists encourage people to reinterpret situations and events in less threatening ways.

Exercise

Exercise also can help to reduce stress. It may seem contradictory to say that both relaxation and exercise reduce stress, but exercise does help people to relax (Mobily, 1982). Suppose you are tense about something that you have to do tomorrow. Your sympathetic nervous system becomes highly aroused in preparation for that event, and there is nothing you can do about it. Under those conditions, the best approach may be to work off some of your excess energy through exercise and then relax afterward.

Regular exercise also prepares people for the unexpected. People in good physical condition react less strongly than other people do to stressful events (Crews & Landers, 1987). An event that would elevate the heart rate enormously in other people elevates it only moderately in a person who has been exercising regularly.

Distraction

Another powerful blunting strategy is distraction (Cioffi, 1991). For example, many people find that they can reduce dental or postsurgical pain by playing video games or by watching comedies on television. The Lamaze method teaches pregnant women to suppress the pain of childbirth by concentrating on breathing exercises.

How effective a distraction is depends partly on whether a person believes that it will help. In one experiment, college students were asked to hold their fingers in ice water until the sensation became too painful to endure (Melzack, Weisz, & Sprague, 1963). Some of them listened to music of their own choice and were told that listening to music would lessen the pain. Others also listened to music but were given no suggestion that it would ease the pain. Still others heard nothing but were told that a special “ultrasonic sound” was being transmitted that would lessen the pain. The group that heard music and expected it to lessen the pain tolerated the pain better than either of the other two groups. Evidently, neither the music nor the suggestion of reduced pain is as effective as both are together.

Distraction also helps us to cope with stress that is not painful. People who are concentrating on a difficult task find it helpful to take a break once in a while. They may go to a movie, read something entertaining, play a round of golf, or just daydream. Furthermore, trying to find the humor in a stressful situation often provides an effective distraction.

SOMETHING TO THINK ABOUT

Many experiments report that a placebo alone serves as an effective painkiller for certain patients. Why might that be? *

THE MESSAGE *Emotions and the Rest of Psychology*

We discuss various aspects of psychology in different chapters—cognition, motivation, emotion, and so forth. This seems a reasonable way to organize a psychology textbook, but our experiences do not divide up so neatly into separate parts. As you have seen in this chapter, our emotions are closely linked to our biology, our motivations, and our memory and cognitions. Any factor that changes one aspect of our experience—say, cognition—will have rippling effects on emotions and motivations as well.

S U M M A R Y

- * **Coping styles.** People’s strategies for dealing with stress fall into two major categories. Monitoring is an attempt to focus on the source of the problems; blunting is an attempt to distract oneself and to reduce the distress without addressing the problem. (page 467)
- * **Prediction and control.** Events are generally less stressful when people think they can predict or control them. (page 468)
- * **Inoculation.** Someone who has experienced a mildly stressful experience is less stressed than other people are by a later, more intense version of the same experience. (page 469)
- * **Social support.** Support and encouragement from friends and family help to alleviate stress. Many people cope with problems by talking with other people who have dealt with similar problems, such as members of self-help groups. (page 469)
- * **Beliefs.** A belief in one’s capacity to succeed may help to reduce stress, even if this belief is not entirely accurate. (page 470)
- * **Relaxation and exercise.** One way of coping with stress is to find a quiet place, relax the muscles, and eliminate distracting stimuli. Exercise can be a helpful way of channeling nervous energy and enabling oneself to relax. (page 471)
- * **Distraction.** Distracting a person’s attention from the source of the stress helps to reduce the stress. (page 472)

Suggestion for Further Reading

Pennebaker, J. W. (1990). *Opening up*. New York: William Morrow and Company. A description of the stress-relieving values of discussing your most painful experiences, either with other people or to yourself in writing.

Terms

- monitoring** attending carefully to the stressful event and one's reaction to it and trying to take effective action (page 467)
- blunting** trying to avoid the stressful event or at least avoid thinking about it (page 467)
- inoculation** protection against the harmful effects of stress by earlier exposure to smaller amounts of it (page 469)

Answers to Concept Checks

5. Your roommate's radio would be more disruptive. You can turn your own radio on or off, switch stations, or reduce the volume. You have no such control over your roommate's radio (unless your roommate happens to be very cooperative). (page 468)
6. Practice giving your speech to a small group of friends. If possible, practice giving the speech in the room where you will ultimately deliver it. (page 469)

Web Resources

Stress Assess

wellness.uwsp.edu/Health_Service/services/stress.shtml

Evaluate your current stress sources, distress symptoms, and lifestyle behaviors. A personalized assessment includes strategies to counteract stress.

How To Fight & Conquer Stress

www.coolware.com/health/medical_reporter/stress.html

Material from the Rose Men's Health Resource defines stress, examines its positive and negative aspects, identifies some major life stressors, and provides tips on how to combat the negative effects of stress.

Personality



Geronimo with His Spirit by Frederick Brown (1984)

13

★ MODULE 13.1

Major Approaches to Personality

Conceptions of Personality

The Psychodynamic Approach

The Trait Approach

The Learning Approach

Humanistic Psychology

Carl Rogers and the Goal of Self-Actualization /

Abraham Maslow and the Self-Actualized Personality

★ MODULE 13.2

Grand Theories of Personality

Sigmund Freud and Psychoanalysis

- *Psychoanalysis and the Unconscious / Further Examples of Casual Use of Evidence / Stages of Psychosexual Development in Freud's Theory of Personality / Freud's Description of the Structure of Personality / Defense Mechanisms Against Anxiety / Manifestations of the Unconscious in Everyday Life / FREUDIAN SLIPS: WHAT'S THE EVIDENCE? / Freud's Legacy / Neo-Freudians*

Carl Jung and the Collective Unconscious

Alfred Adler and Individual Psychology

Adler's Description of Personality / Adler's View of Psychological Disorders / Adler's Legacy

★ MODULE 13.3

Personality Traits

Personality Traits and States

The Search for Broad Personality Traits

The "Big Five" Personality Traits / Pros and Cons of the Big Five Description

The Origins of Personality

★ MODULE 13.4

Personality Assessment

Standardized Personality Tests

Objective Tests of Personality

The Minnesota Multiphasic Personality Inventory /

Revisions of the Test / The Generalizability of the

MMPI / Detection of Deception on the MMPI and

Elsewhere / Uses of the MMPI / The 16-PF Test

Projective Techniques

The Rorschach Inkblots / The Thematic Apperception Test / Less Successful Projective Techniques

Prospects for New Personality Tests

Uses and Misuses of Personality Tests

Several thousand people have the task of assembling the world's largest jigsaw puzzle, which contains more than one trillion pieces. Connie Conclusionjumper examines 20 pieces very closely, stares off into space, and announces, "When the puzzle is fully assembled, it will be a picture of the Houston Astrodome!" Prudence Plodder says, "Well, I don't know what the whole puzzle will look like, but I think I've found two little pieces that fit together."

Which of the two is making the greater contribution to completing the puzzle? We could argue either way. Clearly the task would require an enormous number of little, unglamorous accomplishments like Prudence's. But if Connie is right, her flash of insight will be extremely valuable for assembling all the little pieces. Of course, if the puzzle turns out to be a picture of two sailboats on Lake Erie, then Connie will have made us waste time looking for nonexistent connections.

Some psychologists have offered grand theories about the nature of personality. Others have investigated why people with a certain type of personality act the way they do in a specific situation. In this chapter, we shall explore several

methods of approaching personality, ranging from large-scale to small-scale, from theoretical to descriptive.



Characters in ancient plays wore masks to indicate their personalities, and we still look to people's faces as one marker of their personalities—although it is often a misleading cue. Caricature, a popular art style, exaggerates facial expression.

Major Approaches to Personality

What is personality and how does it develop?

What are some effective ways to study personality?

Every individual is virtually an enemy of civilization. . . . Thus civilization has to be defended against the individual. . . . For the masses are lazy and unintelligent. . . . and the individuals composing them support one another in giving free rein to their indiscipline.

—SIGMUND FREUD (1927/1953)

It has been my experience that persons have a basically positive direction. In my deepest contacts with individuals in therapy, even those whose troubles are most disturbing, whose behavior has been most anti-social, whose feelings seem most abnormal, I find this to be true.

—CARL ROGERS (1961)

What makes us tick? What makes us the way we are? Way down deep, are humans mostly good or mostly bad?

The 17th-century philosopher Thomas Hobbes argued that humans are by nature selfish. Life in a state of nature, he said, is “nasty, brutish, and short.” If we are to protect ourselves from one another, we must be restrained by a watchful government.

The 18th-century political philosopher Jean-Jacques Rousseau disagreed. He maintained that humans are basically good and that “civilized” governments are the problem, not the solution. Although he conceded that society could never return to “noble savagery,” he believed that education and government should promote the freedom of the individual. Rational people acting freely, he maintained, would advance the welfare of all.

The debate between those two viewpoints survives in modern theories of personality (Figure 13.1). Some theorists, including Sigmund Freud, have held that people are born with sexual and destructive impulses that must be held in check if civilization is to survive. Others, including Carl Rogers, believed that people will achieve good and noble goals once they have been freed from unnecessary restraints.

Hobbes	Rousseau
Government is required for protection	Humans are good
Humans are selfish	Government is a corrupting influence
Freud	Rogers
Natural impulses are detrimental to society	Natural impulses are noble and good

FIGURE 13.1 Philosophers Thomas Hobbes and Jean-Jacques Rousseau expressed opposing views of human nature. Psychologists Sigmund Freud and Carl Rogers also expressed opposite views. Freud, like Hobbes, stressed the more negative aspects of human nature; Rogers, the more positive or optimistic aspects.

Which point of view is correct? Or does the truth lie somewhere in between? The study of human personality is complicated, and we can easily be tempted to emphasize one aspect of personality to the exclusion of the others.

Conceptions of Personality

The term *personality* comes from the Latin word *persona*, meaning “mask.” In the plays of ancient Greece and Rome, actors wore masks to indicate whether they were comic or tragic characters. Unlike a mask that one can either put on or take off, however, the term *personality* implies something stable. **Personality** consists of *all the consistent ways in which the behavior of one person differs from that of others, especially in social situations*. (Differences among people in their learning, memory, or sensory functions are not generally considered part of personality.)

The ancient Greeks believed that personality depended on which of four different “humors” predominated in a person’s body (Figure 13.2). A predominance of yellow bile made people hot tempered. A predominance of black bile made people depressed. An excess of phlegm made people sluggish and apathetic. An excess of blood made people courageous, hopeful, and amorous. The ancient Greek theory persists in the English language in such terms as *phlegmatic* and *melancholic* (literally, “black-bile-ic”).

Today, although we no longer believe in the four humors, we do believe that personality is influenced by other chemicals, such as hormones and neurotransmitters. It is further influenced by our experiences, including our observation and imitation of other people’s behavior.



FIGURE 13.2 According to the second-century Greek physician Galen, people's personalities depended on four humors. These were, clockwise from top left: sanguine, melancholic, phlegmatic, and choleric. Galen's theory was popular in medieval Europe.

In this brief first module, we shall quickly survey the major approaches to the study of personality: the *psychodynamic* approach (see the second module), the *trait* approach (see the third module), the *learning* approach (already discussed in the social learning module of Chapter 6), and the *humanistic* or *phenomenological* approach. The humanistic approach will receive a little more attention here because it does not have a module of its own.

The Psychodynamic Approach

Sigmund Freud's theory was the first of several psychodynamic theories. A **psychodynamic theory** *relates personality to the interplay of conflicting forces within the individual, including some that the individual may not consciously recognize*. That is, we are being pushed and pulled by internal forces that we do not fully understand.

The nature of these forces varies from one theory to another. For example, Freud (Figure 13.3) emphasized sexual motivation. Suppose you go to visit someone but you forget your umbrella and leave it behind. You didn't "just forget," Freud would say; perhaps you secretly love the person you were visiting, and by leaving your umbrella you create an excuse to go back and visit again—"to pick up the umbrella." Your secret love might be so secret that you are not even aware of it yourself; it is an unconscious feeling but nevertheless it influences your behavior.

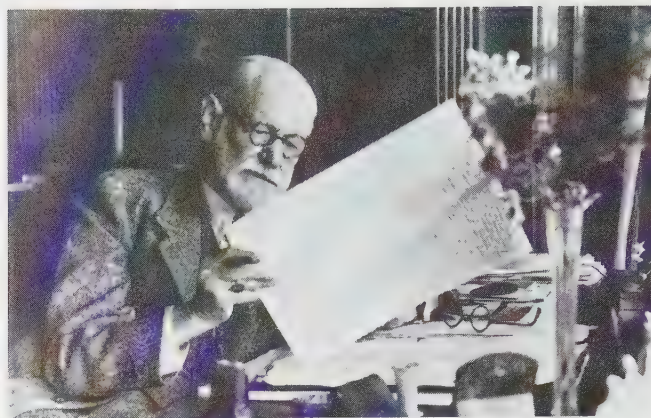


FIGURE 13.3 Sigmund Freud offered interpretations of dreams, slips of the tongue, psychological disorders, and other behaviors that people had previously considered "random" or "unexplainable." According to Freud, even apparently purposeless behaviors reveal the influence of unconscious thoughts and motivations. Freud's theories have had an immense influence on many people, not just on psychologists. For example, many literary commentators have offered psychoanalytic interpretations of novels and plays. Most psychological researchers, however, are skeptical of Freud's approach.

Carl Jung proposed that your behavior is guided by a different kind of unconscious influence: If you dream about a beetle, for example, it might be because beetles have played an important part in human mythology at least dating back to the ancient Egyptians. If you draw a circle inside a square inside a circle, you are also recreating symbolism that has appeared in various cultures throughout the world. Somehow, Jung said, the experiences of your ancestors have caused you to be born with a predisposition to develop certain thoughts and images. (Jung accepted the theory of inheritance of acquired characteristics, a theory that biologists have thoroughly discredited.)

Alfred Adler suggested that people are guided by their ambitions—that, above all else, we are striving for success or superiority, or at least a belief in our potential for success. However, we are sometimes unaware of our strategies for maintaining this belief. If you marry someone who is likely to thwart your ambitions, perhaps your underlying motivation is to be able to maintain an illusion: "I could have been a great success if my husband/wife hadn't prevented me." Failure to study can have a similar motivation: "I could have done well on this test, but my friends talked me into going out to a party the night before."

Although Freud, Jung, Adler, and other theorists differ strongly about the nature of people's underlying motivations, they agree that people become more effective in their lives by becoming more aware of their thoughts and feelings. In short, the psychodynamic approaches claim that many of the influences behind anyone's personality are hidden and that much of our behavior is the result of competing motivations.

The Trait Approach

Whereas psychodynamic theorists try to explain the underlying basis for personality, many contemporary researchers concentrate on simply describing the differences among personalities. The point of the **trait approach to personality** is that *people have consistent personality characteristics that can be measured and studied*. If you try to describe someone you know, you will almost certainly list some personality traits, such as “she’s friendly, honest, has a good sense of humor, but sometimes gets too upset by criticism.” Psychologists try to make these descriptions more accurate and systematic. It would be helpful, for example, to specify *how* friendly or honest this person is, compared to others, or to specify in which situations he or she is friendly or honest.

Let’s start with this example: Some people match their gender stereotypes rather closely, being either very “masculine” men or very “feminine” women. According to the usual or stereotypical meanings of the terms, *masculinity* includes ambitiousness, self-assertiveness, and an interest in sports. *Femininity* includes an enjoyment of children, an enjoyment of beautifying the house and garden, and a tendency to be sympathetic to and understanding of other people.

We could therefore identify masculinity and femininity as personality traits. One of the tasks for researchers, then, is to determine whether these are really just two ends of a single continuum (just as outgoingness is the opposite of isolation), or whether they are separate dimensions. Sandra Bem (1974) proposed that it is possible to be *high in both masculinity and femininity, or flexible enough to switch between masculinity and femininity depending on the situation*. She defined this trait as **androgyny** (from the Greek roots *andr-* meaning “man” and *gyne-* meaning “woman”). According to Bem, androgynous people are not limited by one stereotype or the other and can combine masculine strengths such as ambition with feminine strengths such as sympathy for the needs of others.

Table 13.1 presents part of a checklist of masculine

and feminine traits. In the original scoring system, you would be considered androgynous if you checked about the same number of masculine items as feminine items. However, one way to be *equal* in both categories is to be very low in both. It is hard to see any advantage in being unassertive, unambitious, indifferent to children, unsympathetic to other people, and low in all the other masculine and feminine characteristics. Consequently, most investigators now define androgyny as a personality that is high in both masculinity and femininity.

Consider a second example of personality traits: Do you think that your successes and failures depend mostly on your own efforts or mostly on circumstances beyond your control? Presumably, we all agree that both are important, but people differ in how much they emphasize one or the other. People who *believe they are largely in control of their lives* are said to have an **internal locus of control**. Those who *believe they are controlled mostly by external forces* are said to have an **external locus of control** (Rotter, 1966). Table 13.2 lists some items from a questionnaire designed to measure locus of control. After you complete these items, check answer A, page 484.

Generally, people with an internal locus of control like to choose tasks where they believe they can control the outcome, and then they persist at these tasks. At the end, they take the credit for their successes and the blame for their failures. People with an external locus of control are more likely to feel helpless (Lefcourt, 1976).

TABLE 13.1 Sample Items from the Bem Sex-Role Inventory

MASCULINE ITEMS	FEMININE ITEMS
Ambitious	Affectionate
Assertive	Cheerful
Competitive	Compassionate
Makes decisions easily	Loves children
Self-reliant	Loyal
Willing to take risks	Sympathetic

SOURCE: Bem, 1974, p. 156.

TABLE 13.2 Sample Items from the Internal-External Scale

For each item, choose the statement you agree with more.

1. a. Without the right breaks, one cannot be an effective leader.
b. Capable people who fail to become leaders have not taken advantage of their opportunities.
2. a. Becoming a success is a matter of hard work; luck has little or nothing to do with it.
b. Getting a good job depends mainly on being in the right place at the right time.
3. a. As far as world affairs are concerned, most of us are the victims of forces we can neither understand nor control.
b. By taking an active part in political and social affairs, people can control world events.
4. a. Many times I feel that I have little influence over the things that happen to me.
b. It is impossible for me to believe that chance or luck plays an important role in my life.

SOURCE: Rotter, 1966, pp. 11–12.

**TRY IT
YOUR-
SELF**

CONCEPT CHECK

- I. Who would be more likely to buy lottery tickets—someone with an internal or external locus of control? (Check your answer on page 484.)

Androgyny and locus of control are just two personality traits; obviously, we could define and measure a great many more. We describe the sizes of objects in terms of three dimensions of space (length, width, and height) and one dimension of time. How many dimensions would we need to describe people's personalities? We shall explore that question later in this chapter.

The Learning Approach

How does someone develop a personality trait, such as androgyny or locus of control? Genetics and other biological factors no doubt exert some influence, but specific behaviors must be learned. Indeed, some psychologists have argued that the whole concept of general personality traits is vastly overrated, and that most of our personality is learned on a situation-by-situation basis (Mischel, 1973, 1981). For example, you might consider yourself honest because you would always return a lost wallet to its owner and because

you would never cheat on your income taxes; nevertheless, you may find yourself telling each of your dating partners, "You are the only one I have ever loved." So how useful is it to say that you are an honest person, or even that you are "more honest than 80% of other people"? It might be more useful to describe specific behaviors, such as honesty about returning a lost wallet or dishonesty toward romantic partners, or conscientiousness in particular situations, or friendliness in particular situations (Mischel & Shoda, 1995). And, presumably, these specific behaviors are learned.

The Social Learning module of Chapter 6 described some ways in which we learn our personality. We learn much by imitation or by vicarious reinforcement and punishment. That is, we copy behaviors that we know led to success for other people but avoid behaviors that led to failure for others.

Let's illustrate this idea by applying this approach to masculinity, femininity, and androgyny. One major part of those traits is **gender role**, *the pattern of behavior that each person is expected to follow because of being male or female*. A gender role is the psychological aspect of being male or female, as opposed to sex, which is the biological aspect. We know that gender role is at least partly learned, because certain aspects of it vary strikingly among cultures (Figure 13.4). For example, some cultures define cooking as "women's work" and others define it as "men's work." Men wear their hair short in some cultures and long in others.



FIGURE 13.4 Gender roles vary greatly from one culture to another, and even from one time period to another within a single culture. Here, a male Palestinian farmer near Jerusalem (upper left) and a woman in Vietnam (upper right) plow the fields. Men in Dacca, East Pakistan (lower left), and a Hmong woman in Thailand (lower right) do the wash.

When we say that children learn their gender role, we do not necessarily mean that anyone teaches it to them deliberately or intentionally. For example, most adults do *not* teach boys that they are supposed to fight with one another, and adults usually do what they can to stop the fighting. Nevertheless, little boys tend to be bossy and aggressive, especially when they think no adults are watching (Powlishta & Mac-coby, 1990). Some of that aggressiveness probably arises from biological tendencies, but boys may have also learned some of it by following adults' examples. Boys tend to imitate men, and girls tend to imitate women. In one experiment, children watched adults choose between an apple and a banana. If all the men chose, say, the apple and all the women chose the banana, the boys who were watching wanted an apple and the girls wanted a banana (Perry & Bussey, 1979). The choice of fruit is, in itself, trivial, but the study shows the potential for influence on more important behaviors.

Perhaps even more importantly, children learn gender roles from other children. Children have a "playground culture" of their own; each cohort teaches the slightly younger set what is expected of them, and in many ways the peer group has a stronger influence than parents do (Harris, 1995). Even parents who try to raise their sons and daughters to be nonsexist find that their children come back from the playground with strong prejudices about what boys do and what girls do (Figure 13.5).



FIGURE 13.5 Children learn their gender roles partly by imitating adults, but they probably learn more from other children.

In short, the learning approach focuses on more specific behaviors than does the trait approach and attempts to relate specific behaviors to specific experiences. Some of these are the person's own experiences and some are those of others whom the person has imitated.

Humanistic Psychology

Another general perspective on personality, **humanistic psychology**, deals with consciousness, values, and abstract beliefs, including spiritual experiences and the beliefs that people live by and die for. According to humanistic psychologists, personality depends on what people believe and how they perceive the world. If you *believe* that a particular experience was highly meaningful, then it *was* highly meaningful. A psychologist can understand your behavior only by asking you for your own evaluations and interpretations of the events in your life. (In theology, a *humanist* glorifies humans, generally denying or at least giving little attention to a Supreme Being. The term *humanistic psychologist* implies nothing about a person's religious beliefs.)

Humanistic psychology emerged in the 1950s and 1960s as a protest against both behaviorism and psychoanalysis, which were the dominant viewpoints in psychology at that time (Berlyne, 1981). Behaviorists and psychoanalysts often emphasize the less noble, or at least morally neutral, aspects of people's thoughts and actions, whereas humanistic psychologists see people as essentially good and striving toward perfection. Also, behaviorism and psychoanalysis, despite their many differences, are both rooted in *determinism* (the belief that every behavior has a cause) and in *reductionism* (the attempt to explain behavior in terms of its component elements). Humanistic psychologists reject these attempts to explain behavior in terms of its parts or causes. They claim that people make deliberate, conscious decisions about what to do with their lives. People can decide to devote themselves to a great cause, to sacrifice their own well-being, and to risk their lives. To the humanistic psychologist, it is fruitless to ascribe such behavior to past rewards and punishments or to unconscious thought processes.

Humanistic psychologists generally study the special experiences of a given individual, as opposed to seeking means or medians for large representative groups. For example, humanistic psychologists study growth experiences—the moments that people identify as points of transition, when they may say, "Aha! Now I have become an adult," or "Now I have truly committed my life to this goal" (Frick, 1983). They also study **peak experiences**, moments in which a person feels truly fulfilled and content. Some people report that they "feel at one with the universe" when they hear "thrilling" music, or take part in an emotional religious ceremony, or achieve a great accomplishment.

Carl Rogers and the Goal of Self-Actualization

Carl Rogers, an American psychologist, studied theology before turning to psychology, and the influence of those early studies is apparent in his view of human nature. Rogers (Figure 13.6) became probably the most influential humanistic psychologist.

According to Rogers (1980), human nature is basically good. People have a natural drive toward **self-actualization**, which means *the achievement of one's full potential*. According to Rogers, it is as natural for people to strive for excellence as it is for a plant to grow. The drive for self-actualization is the basic drive behind the development of personality. (Rogers's concept of self-actualization is similar to Adler's concept of striving for superiority. Adler was a forerunner of humanistic psychology.)

Children evaluate themselves and their actions, beginning at an early age. They learn that what they do is sometimes good and sometimes bad. They develop a **self-concept**, *an image of what they really are*, and an **ideal self**, *an image of what they would like to be*. Rogers measured a person's self-concept and ideal self by handing the person a stack of cards containing statements such as "I am honest" and "I am suspicious of others." The person would then sort the statements into piles representing *true of me* and *not true of me*, or arranging them in a continuum from *most true of me* to *least true of me*. (This method is known as a *Q-sort*.) Then Rogers would provide an identical stack of cards and ask the person to sort them into two piles: *true of my ideal self* and *not true of my ideal self*. In this manner, he could determine whether someone's self-concept was similar to his or her ideal self; people who perceive a great discrepancy between the two generally experience distress. Humanistic psychologists try to help people overcome this distress, either by improving their self-concept or by changing their ideal self.

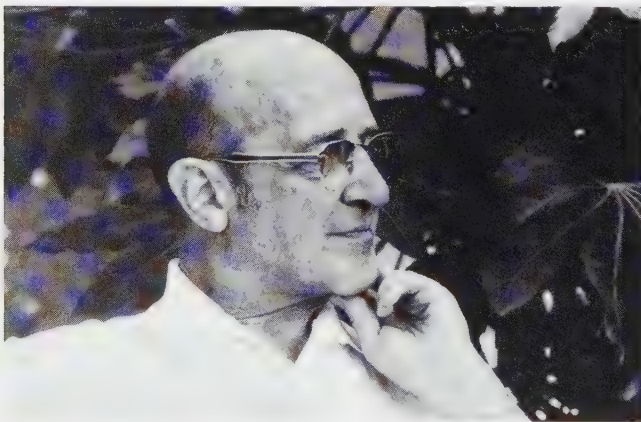


FIGURE 13.6 Carl Rogers maintained that people naturally strive toward positive goals and that they do not need special urging. He recommended that people relate to one another with "unconditional positive regard."

To promote human welfare, Rogers maintained that people should relate to one another with **unconditional positive regard**, a relationship that Thomas Harris (1967) described with the phrase "I'm OK—you're OK." Unconditional positive regard is *the complete, unqualified acceptance of another person as he or she is*, much like the love of a parent for a child. If someone expresses anger, or even a desire to kill, the listener should accept that as an understandable feeling, even while discouraging the person from taking certain possible actions. The listener must convey the message that the other person is inherently good, even though certain actions might be bad. This view resembles the Christian admonition to "hate the sin but love the sinner." The alternative is *conditional positive regard*, the attitude that "I shall like you only if . . ." People who are treated this way may feel restrained about opening themselves to new ideas or behaviors, for fear of losing someone else's esteem.

Abraham Maslow and the Self-Actualized Personality

Abraham Maslow (Figure 13.7), another of the founders of humanistic psychology, proposed that people have a hierarchy of needs, an idea we considered in Chapter 11. The highest of those needs is *self-actualization*, the fulfillment of a person's potential. What kind of person achieves self-actualization, and what is the result of achieving it? Maslow (1962, 1971) sought to describe the self-actualized personality. He complained that psychologists concentrate on disordered personalities, thus reflecting the medical view that health is merely the absence of disease. They seem to assume that all personality is either "normal" (that is, bland) or undesirable. Maslow insisted that personality can differ from the normal in positive, desirable ways.

To determine the characteristics of the self-actualized personality, Maslow made a list of people who, in his opinion,



FIGURE 13.7 Abraham Maslow, one of the founders of humanistic psychology, introduced the concept of a "self-actualized personality," a better-than-merely-normal personality associated with high productivity and enjoyment of life.



FIGURE 13.8 Harriet Tubman, left, one of the people Maslow identified as having a self-actualized personality, was one of the leaders of the Underground Railroad, a system for helping black slaves to escape from the southern states during the period before the Civil War. Maslow described the self-actualized personality by identifying highly productive and admirable people, such as Tubman, and then determining which personality features these people had in common.

had achieved their full potential. His list included people he knew personally as well as figures from history (Figure 13.8). He then sought to discover what these people had in common.

According to Maslow (1962, 1971), people with a self-actualized personality show the following characteristics:

- An accurate perception of reality: They perceive the world as it is, not as they would like it to be. They are willing to accept uncertainty and ambiguity when necessary.
- Independence, creativity, and spontaneity: They follow their own impulses.
- Acceptance of themselves and others: They treat people with unconditional positive regard.
- A problem-centered outlook, rather than a self-centered outlook: They think about how best to solve a problem, not how to make themselves look good. They also concentrate on significant problems, such as philosophical or political issues, not just the petty issues of getting through the day.
- Enjoyment of life: They are open to positive experiences, including “peak experiences.”
- A good sense of humor.

Critics have attacked Maslow’s description on the grounds that, because it is based on his own choice of subjects, it may simply reflect the characteristics that he himself admired. In any case, Maslow paved the way for other attempts to define a healthy personality as something more than personality without disorder.

THE MESSAGE

In Search of Human Nature

You have no doubt heard the parable of the blind people describing an elephant: One feels the tusks and says an elephant is like a smooth rock; another feels the tail and says an elephant is like a rope; and so forth. The full description of an elephant can emerge only from the combination of narratives. Similarly, a full description of personality emerges from a combination of research approaches. Saying that people have personality traits does not necessarily conflict with saying that personality is learned, or that it has unconscious influences (the psychodynamic approach), or that it is largely under conscious control (the humanistic approach).

However, although these approaches are not necessarily in conflict, their advocates are not necessarily in full agreement, either: We began with contrasting quotes about people being either fundamentally bad (Freud) or good (Rogers). Various personality theorists have sharply different views of human nature, and much is at stake. The challenge for personality researchers is to take such global questions about human nature and try to convert them into scientifically testable hypotheses.

SUMMARY

★ *Personality theories as views of human nature.* Personality consists of all the stable, consistent ways that the behavior of one person differs from that of others. Theories of personality are closely related to conceptions of human nature. Some observers believe that human beings are basically hostile and need to be restrained (Hobbes, Freud). Others believe that human beings are basically good and are hampered by restraints (Rousseau, Rogers). (page 477)

★ *Psychodynamic theories.* Several historically influential theories have described personality as the outcome of internal forces of which people are not fully conscious. (page 478)

★ *Trait theories.* Many contemporary researchers try to describe and measure the consistent ways that one person’s personality differs from another’s. (page 479)

★ *The learning approach.* The behaviors that constitute personality are learned. They can be learned through individual experience, or as social learning psychologists emphasize, they can be learned by imitation or vicarious reinforcement and punishment. Because people’s experiences

vary, they can have very specialized characteristics, showing a trait such as honesty in one situation but not in another. Gender roles vary among cultures because of social learning. (page 480)

★ **Humanistic psychology.** Humanistic psychologists emphasize conscious, deliberate decision-making; they oppose attempts to reduce behavior to its elements or to seek explanations in terms of unconscious influences. (page 481)

Suggestion for Further Reading

Maslow, A. H. (1962). *Toward a psychology of being*. Princeton, NJ: Van Nostrand. A good introduction to humanistic psychology.

Terms

personality all the consistent ways in which the behavior of one person differs from that of others, especially in social situations (page 477)

psychodynamic theory a system that relates personality to the interplay of conflicting forces within the individual, including some that the individual may not consciously recognize (page 478)

trait approach to personality the study and measure of consistent personality characteristics (page 479)

androgyny a condition of being high in both masculine and feminine traits, or flexible enough to switch between masculinity and femininity depending on the situation (page 479)

internal locus of control the belief that one is largely in control of the events of one's life (page 479)

external locus of control the belief that external forces are largely in control of the events of one's life (page 479)

gender role the pattern of behavior that each person is expected to follow because of being male or female (page 480)

humanistic psychology a field that is concerned with consciousness, values, and abstract beliefs, including spiritual experiences and the beliefs that people live by and die for (page 481)

peak experiences moments in a person's life when he or she feels truly fulfilled and content (page 481)

self-actualization the achievement of one's full potential (page 482)

self-concept an image of what one really is (page 482)

ideal self an image of what one would like to be (page 482)

unconditional positive regard the complete, unqualified acceptance of another person as he or she is (page 482)

Answer to Concept Check

1. People with an external locus of control would be more likely to buy lottery tickets. Those with an internal locus of control prefer tasks where they can control the outcome. (page 480)

Answer to Other Question in the Text

- A. Choices 1b, 2a, 3b, and 4b indicate internal locus of control; the other choices indicate external locus of control. Your answers to a longer list of such items could more accurately assess your locus of control. (page 479)

Web Resources

Personality and Consciousness

www.wynja.com/personality/theorists.html

This growing site includes papers about Adler, Freud, Jung, Kelly, Lewin, Maslow, Rogers, Skinner, and others. You are invited to submit your own paper(s) for others to read; you will retain the copyright.

Grand Theories of Personality

Is personality rooted in one or two dominant motivations, such as sexuality or the desire for superiority?

What is a “healthy” personality?

The most famous person associated with psychology is surely Sigmund Freud. His influence on society is vast, extending into sociology, literature and the arts, even religion and politics (Figure 13.9). And yet, here we are, about three-fourths of the way through this text on psychology, and until now I have barely mentioned Freud. Why?

The reason is that, although Freud continues to be highly influential outside psychology, his influence within psychology is very much on the decline. According to one psychologist, Frederick Crews (1996), “. . . independent studies have begun to converge toward a verdict that was once considered a sign of extremism or even of neurosis: that there is literally nothing to be said, scientifically or therapeutically, to the advantage of the entire Freudian system or any of its constituent dogmas.”

Think about that: *nothing* to be said in favor of *any* of Freud’s theories. Needless to say, not everyone agrees with Crews. Some psychologists still loyally adhere to Freud’s theories and methods, and even many who do not regard

themselves as Freudians have been influenced by him, perhaps in ways they do not realize. Still, the decline of Freud’s influence is striking. In this module, we shall consider how Freud developed his theories and how he used or misused evidence. We shall also consider the ideas of several other theorists who also developed grand and ambitious theories of personality based on different assumptions.

Sigmund Freud and Psychoanalysis

Sigmund Freud (1856–1939; Figure 13.3), an Austrian physician, would have liked to become a professor of cultural history or anthropology; he wrote several books and articles about those topics in his later years. As a Jew in late 19th-century Austria, however, he knew that he had little chance of becoming a university professor. The only professional careers open to Jews in his time and place were in law, business, and medicine. Freud chose medicine, but without any deep commitment to the field.

Psychoanalysis and the Unconscious

Early in his career, Freud worked with the psychiatrist Josef Breuer, who had been treating a young woman with a fluctuating variety of physical complaints. As she talked with Breuer about her past, she recalled various traumatic, or emotionally damaging, experiences. Breuer, and later Freud also, proposed that recalling these experiences produced **catharsis**, *a release of pent-up emotional tension*, and thereby relieved her illness. However, later scholars who re-examined the medical records of the time found that this woman was not at all cured by this treatment. She ended the treatment and entered a hospital, where she was treated with drugs. Eventually, she recovered, although the cause of her problems and the basis for her recovery remain mysterious (Ellenberger, 1972).

Regardless of whether Breuer’s “talking cure” had been successful, Freud began applying it to his own emotionally disturbed patients. He referred to *his method of explaining and dealing with personality, based on the interplay of conscious and unconscious forces*, as **psychoanalysis**. To this day, psychoanalysts remain loyal to that method and to Freud’s theories.

Psychoanalysis started out as a fairly simple theory: Each of us has an unconscious mind as well as a conscious mind (Figure 13.10). The **unconscious** is *the repository of memories, emotions, and thoughts, many of them illogical, that affect our behavior even though we cannot talk about them*. Traumatic experiences force thoughts and emotions into the unconscious, and the goal of psychoanalysts is to bring those memories to consciousness. Doing so produces catharsis and enables the person to confront irrational and self-defeating impulses.



FIGURE 13.9 The fame of Sigmund Freud far exceeds that of any other psychologist. His picture has even been placed on the Austrian 50-schilling bill.